

probenecid and sulfinpyrazone is attenuated.

Oral anticoagulants: Non-steroidal anti-inflammatory drugs may increase the effects of anticoagulants such as warfarin.

Platelet aggregation inhibitors and selective serotonin reuptake inhibitors: Increased risk of gastrointestinal haemorrhage.

Anti-diabetic medicinal products: Ibuprofen enhances the hypoglycemic effect of oral antidiabetic medication and insulin. It may be necessary to adjust the dosage.

Aminoglycosides: NSAID's may decrease the excretion of aminoglycosides.

Diuretics: The efficacy of furosemide and thiazide diuretics can be decreased, probably due to sodium retention, related to an inhibition of prostaglandin synthesis in the kidneys.

Anti-hypertensive agents: A weakening of the efficacy of anti-hypertensive drugs is to be expected. As a result, concomitant treatment with NSAIDs and ACE inhibitors or beta-blocker agents may increase the risk of acute renal insufficiency.

Histamine H2-receptor antagonists: A clinically meaningful interaction of ibuprofen with cimetidine or ranitidine has not been established.

Digoxin: The plasma concentration of digoxin may increase.

Phenytoin: The plasma concentration of phenytoin may increase.

Lithium: The plasma concentration of lithium may increase.

Methotrexate: Increased methotrexate toxicity.

Zidovudine: The concomitant use of zidovudine and ibuprofen increases the risk of haemarthrosis and haematoma in HIV (+) haemophilic patients.

Tacrolimus: The risk of nephrotoxicity may be increased with concomitant use of tacrolimus and ibuprofen.

Voriconazol and Fluconazol: Concomitant use of voriconazole, fluconazole and ibuprofen may lead to an increase in the exposure and plasma concentration of ibuprofen.

Mifepristone: Co-administration of NSAIDs may increase NSAID exposure. A decrease in the efficacy of mifepristone can occur theoretically due to the anti-prostaglandin properties of NSAIDs. Studies suggest that co-administration of ibuprofen on the same day prostaglandins are administered (or as needed), has no adverse influence on the action of mifepristone or on the clinical efficacy of the pregnancy termination.

Baclofen: Increased baclofen toxicity.

Quinolones: Co-administration of NSAIDs may increase the risk of seizures.

Cyclosporine: The toxic effect on kidneys may increase.

Herbal extracts: Ginkgo biloba may potentiate the risk of bleeding associated with NSAIDs.

Interactions with diagnostic tests:

- Bleeding time (bleeding time may be prolonged until 1 day after discontinuation of therapy)
- Serum glucose concentration (may decrease)
- Creatinine clearance (may decrease)
- Hematocrit or hemoglobin (may decrease)
- BUN, serum creatinine concentrations and kalemia (may increase)
- Liver function test (increase in transaminases).

Pregnancy and Lactation

Pregnancy
Inhibition of prostaglandin synthesis may adversely affect pregnancy and/or embryo-fetal development. Data from epidemiological studies indicates an increased risk of miscarriage, as well as cardiac malformations and gastroschisis following the use of prostaglandin synthesis inhibitors in early pregnancy. It is believed that the risk is in directly proportional with the dose and duration of therapy. In animal studies, it has been shown that administration of prostaglandin synthesis inhibitors leads to increased pre- and post-implanted loss and to embryo fetal lethality. Furthermore, increased incidences of various malformations, including cardiovascular malformations, have been reported in animals receiving prostaglandin synthesis inhibitors during the organogenesis phase.

During the first and second trimesters of pregnancy, Spedifen should only be given if absolutely necessary. If Spedifen is used by women trying to get pregnant or if it is used during the first or second trimester of pregnancy, the dose should be kept as low and the duration of treatment as short as possible.

Oligohydramnios/Neonatal renal failure/Ductus arteriosus constriction

The use of NSAIDs in the 20th week of pregnancy or later may cause fetal renal dysfunction, leading to oligohydramnios and, in some cases, neonatal renal impairment. These undesirable effects are seen, on average, after days to weeks of treatment, although oligohydramnios has been infrequently reported as soon as 48 hours after NSAID treatment initiation. Oligohydramnios is often, but not always, reversible with treatment discontinuation. Complications of prolonged oligohydramnios may, for example, include limb contractures and delayed lung maturation. After market introduction, invasive procedures such as exchange transfusion or dialysis were required in some cases of impaired neonatal renal function. In addition, ductus arteriosus constriction has been reported after the treatment in the second trimester, which resolved in most cases after discontinuation of treatment. Consider ultrasound monitoring of amniotic fluid and fetal heart if the treatment with Spedifen extends beyond 48 hours. Discontinue Spedifen if oligohydramnios or ductus arteriosus constriction occurs and perform a follow up examination according to clinical practice. Spedifen is contraindicated during the third trimester of pregnancy. All prostaglandin synthesis inhibitors can:

- expose the fetus to the following risks:
 - Cardiopulmonary toxicity (with premature closure of the ductus arteriosus and pulmonary hypertension);
 - Kidney dysfunction, which may progress to renal failure with oligohydramniosis.
- expose mother and child to the following risks:
 - Possible prolongation of bleeding time, an anti-aggregating effect, which may occur even at very low doses;
 - Inhibition of uterine contractions, resulting in delayed or prolonged labour.

Lactation

NSAIDs pass into breast milk. As a precaution, Spedifen should therefore not be taken by breast-feeding women. If treatment is essential, the baby should be switched over to bottle feeding.

Fertility

The use of ibuprofen may affect female fertility and is therefore not recommended in women who wish to become pregnant. In women who have difficulty getting pregnant or who are being tested for infertility, discontinuation of ibuprofen should be considered.

Side Effects

Frequency: Very common (≥1/10), common (≥1/100, <1/10), uncommon (≥1/1,000, <1/100), rare (≥1/10,000, <1/1,000), very rare (<1/10,000), unknown (it cannot be defined on the basis of the

available data).

Haemolymphopoietic system disorders
Rare: Haematological effects such as agranulocytosis, thrombocytopenia, neutropenia, aplastic anaemia, haemolytic anaemia.
Unknown: Anaemia.

Immune system disorders
Uncommon: Allergic reaction.
Rare: Lupus erythematosus syndrome, aseptic meningitis in patients suffering from an autoimmune disease such as, e.g., lupus erythematosus, autoimmune haemolytic anaemia, anaphylaxis.
Unknown: Anaphylactic shock.

Psychiatric disorders
Uncommon to common: Depression, anxiety, confusional state.
Very rare: Psychotic states.

Nervous system disorders
Common: Headache, dizziness.
Uncommon to common: Effects on the central nervous system such as limitation of the ability to react (in particular in interaction with alcohol), drowsiness.
Rare: Paraesthesia.
Eye disorders
Uncommon to common: Vision disorder. Vision disorders are normally reversible with discontinuation of treatment.
Rare: Toxic amblyopia, optic neuritis.
Unknown: Papilloedema.

Ear and labyrinth disorders
Uncommon to common: Ear disorder, hypoacusis.

Heart disorders
Unknown: Heart failure, Kounis syndrome.

Vascular disorders
Unknown: Arterial thrombosis, hypertension, hypotension.

Respiratory, thoracic and mediastinal disorders
Uncommon: Bronchospasm, asthma, aggravated asthma, dyspnoea.
Rare: Risk of acute pulmonary oedema in patients with heart failure.
Unknown: Throat irritation.

Gastrointestinal disorders
Very common: Dyspepsia, diarrhoea.
Common: Gastrointestinal side effects such as nausea, abdominal fullness, heartburn, abdominal pain, anorexia, constipation, flatulence, vomiting, erosive gastritis, blood loss from the rectum (up to anaemia).
Uncommon: Peptic ulcer, gastrointestinal haemorrhage, melena, gastritis.
Rare: Ulceration or perforation in the gastrointestinal tract with haemorrhages, haematemesis, ulcerative stomatitis, aggravated colitis, aggravated Crohn's disease.

Hepatobiliary disorders
Rare: Liver disorders, liver failure.
Unknown: Liver injury, hepatitis, jaundice.

Skin and subcutaneous tissue disorders
Common: Hypersensitivity reactions such as urticaria, purpura, itching, rash.
Uncommon: Angioedema.
Very rare: Bullous rashes, severe skin reactions (including erythema multiforme, exfoliative dermatitis, Stevens-Johnson syndrome, toxic epidermal necrolysis), photosensitivity reaction.
Unknown: Aggravation of skin reactions, drug reaction with eosinophilia and systemic symptoms (DRESS syndrome), acute generalised exanthematous pustulosis (AGEP).

Kidney and urinary tract diseases
Rare: Haematuria, renal papillary necrosis, interstitial nephritis, renal function disorders with oedema formation.
Very rare: Acute renal impairment.

Systemic disorders and conditions related to the site of administration
Unknown: Oedema.

Investigations
Rare: Abnormal liver function test.
Unknown: Abnormal renal function test.

Symptoms and Treatment of Overdose

Signs and symptoms
Common symptoms of overdose include nausea, vomiting, dizziness, drowsiness, tremors; Less common are headache, tinnitus, ataxia, tachycardia, miosis and reversible increases in transaminases, bilirubin, and thrombocytopenia. Prolonged use at doses higher than those recommended may cause severe hypokalaemia and renal tubular acidosis. Symptoms may include reduced level of consciousness and generalised weakness. Severe symptoms are rare and include unconsciousness (coma), metabolic acidosis, seizures, and acute renal insufficiency; also, apnea in children < 2 years. From 400 mg/kg, severe symptoms are possible, although doses up to 60 g can be easily tolerated and doses up to 100 g have not been fatal. In elderly people and small children, those suffering from liver or kidney disease or from chronic alcohol abuse, severe symptoms may be experienced, even at lower doses.

Treatment
From a dose of > 200 mg/kg (small child) or 20 g (adult), decontamination with activated charcoal (1 g per kg body weight as aqueous suspension administered once orally). In case of excessive overdose use gastric lavage followed by activated charcoal within 1 hour of intake. Medical monitoring from 300 mg/kg and in all patients at increased risk. Duration of monitoring: 4 hours, with slow-release preparations 12 hours. Laboratory tests: Transaminases, creatinine, bilirubin; in symptomatic patients additionally blood gas analysis, electrolytes, platelets.

Effects on Ability to Drive and Use Machine Spedifen may have an effect on the ability to drive and use machines due to potential undesirable effects

Preclinical Safety Data

Mutagenicity
In vitro and in vivo studies (bacteria, human lymphocytes) for mutagenicity revealed no evidence of any mutagenic effects of ibuprofen. In studies on the tumorigenic potential of ibuprofen in rats and mice, no evidence of carcinogenic effects of ibuprofen was found. Reproductive toxicity
Experimental studies in two animal species have shown that ibuprofen crosses the placenta; however, there was no indication of any teratogenic effects.

Storage Condition: Store at temperature not exceeding 30°C.
Shelf Life: 3 years
Pack size: 30 tablets per box

Composition
Active ingredient: Ibuprofen (as L-arginine salt).
Excipients: L-arginine, sodium hydrogen carbonate, crospovidone, magnesium stearate, hypromellose (Methocel 5cP), sucrose, titanium dioxide, macrogol 4000.
MANUFACTURER:
ZAMBON S.p.A
Via della Chimica, 9 36100 Vicenza, ITALY

Revision of the package: January 2026

6294989 E03.1024 IS041 BV-B (cp. 9/9,8)

Zambon 1905		Property of Zambon S.p.A Via Lillo del Duca 10 20091 Bresso / Milano		PRODOTTO / BRAND: IS SPEDIFEN 400 MG CPR - MALESIA		LATO STAMPA BIANCA/VOLTA (SIDE PRINT FRONT/REAR): LATO RETRO / BACK SIDE	
CODICE / ITEM: 6294989		ED. / VERSION N.: E03.1024		DATA / DATE: 13/01/2026		CONFORME AL MASTER LAYOUT - NOTE: YES	
DIMENSIONI / SIZE: 150x560/280		FORM./TECH.CODE/MASTER LAYOUT: IS041 BV-B		COLORI / COLORS NO: 2		BRILLE: N/A	
COD. CLIENTE / CUSTOMER CODE: N/A		PHARMA CODE: 92 (011101)		EDGE CODE: N/A		FONT/CORPO MIN./INTERLINEA (FONT/MIN. SIZE/SPACING): HELVETICA NEUE Corpo 9 pt. - Interlinea 9,8 pt. ISTRUZIONI FORNITORE / SUPPLIER INSTRUCTION: ISTRUZIONE PIEGATA A META' CON DIMENSIONI FINALI MM 150x280 E CODICI LAETUS VISIBILI ALL'ESTERNO	
STABILIMENTO / PLANT: VICENZA		PACKAGING LINE: LINEA 4					
CHANGE DESCRIPTION: NUOVI TESTI / NEW TEXT							